



## جوستاف روسي انترناشيونال - مصر (EPE) \* طلب علاج أول مرة / متردد

إسم المريض رباعياً : نادية حسين ابوالعلا محمد  
المحافظة : القاهرة الحي / المدينة : مدينه السلام  
رقم البطاقة : 27212110101561 رقم التليفون : 01013912023  
العمر (تاريخ الميلاد) : 11-12-1972 المهنة : بدون عمل  
العنوان : .....  
تاريخ تسجيل الإستمارة : 26-09-2026

### الإقرار

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك  
الإحصائي الاجتماعي  
التوقيع: (.....) المقر بما فيه  
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط ثاني مثبتات  
الاروماتيز [14161]  
قسم العلاج :  
الاورام

التعليق على التشخيص : .....

### تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني  
الاسم : د/ندى حسن  
الاسم : د/نرمين رمضان  
الوظيفة : .....  
الوظيفة : .....  
الوظيفة : .....  
التوقيع : .....  
التوقيع : .....  
التوقيع : .....  
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس  
وتلتزم جهة العلاج بلائحة المجالس الطبية.  
كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.  
تعتمد صحة البيانات المرفقة  
رئيس اللجنة ومدير  
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

### أعضاء اللجنة

أ.د. / ..... أ.د. / ..... أ.د. / ..... د. / ..... د. / .....

### قرار اللجنة

التشخيص النهائي : .....  
خطة العلاج : .....  
المستشفى : .....  
جهة العلاج : .....  
ملاحظات : .....  
قسم : .....  
نفقات : .....

ختم  
شعار الجمهورية

مدير عام  
مديرية الشؤون الصحية

رئيس  
المجلس الطبي المتخصص

Name: نادية حسين ابوالعلا محمد  
National ID: 27212110101561



## تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :  
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :  
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

الوقت :

--:--

التاريخ :

05-07-2026

الطبيب المعالج :  
د/شيماء لاشين

جمهورية مصر العربية  
بطاقة تحقيق الشخصية



تاريخه  
حسن ابو العلا محمد  
مسكن العبد القديم ب ٣٩ م ١  
اول السلام - القاهرة

١٩٧٢/١

٢٧٢١٢١١٠١٠١٥٦١

٢٧٢١٢١١٠١٠١٥٦١



٢٠٢١/٠٣

٣٧٣١٢١١-١٠١٥٦١

ر.ب. منزل

أرضية

مسفحة

أشجار

الخطوط الحدودية حتى ٢٠٢٨/٠٢/٠٢



AL-AZHAR UNIVERSITY

BABEL-SHE'RIYA UNIVERSITY  
HOSPITAL

CARDIOLOGY DEPARTMENT  
ECHOCARDIOGRAPHY UNIT



جامعة الأزهر

مستشفى باب الشعرية الجامعي

قسم القلب والأوعية الدموية

وحدة الموجات فوق الصوتية على القلب

### Transthoracic Echocardiography

| Name | ناديه حسين أبو العلا | Age | 52 y | Date | 17/08/2025 |
|------|----------------------|-----|------|------|------------|
|------|----------------------|-----|------|------|------------|

|        |     |              |       |      |          |
|--------|-----|--------------|-------|------|----------|
| LV EDD | 4.6 | 3.7 - 5.7 cm | FS    | 43 % |          |
| LVESD  | 2.6 | 2 - 4 cm     | EF    | 75 % | ≥ 52 %   |
| IVSD   | 0.8 | 0.7 - 1.1 cm | Aorta | 3.5  | 2 - 4 cm |
| LVPWD  | 0.8 | 0.7 - 1.1 cm | LA    | 3.7  | 2 - 4 cm |

➤ Left ventricle:

- Normal LV internal systolic dimensions with normal LV systolic function.
- Grade I of LV diastolic dysfunction.
- No resting RWMA.

➤ Left atrium and aortic root:

- Normal LA dimension. Normal aortic root dimensions.

➤ Right side of the heart:

- Normal RV and RA dimensions with normal RV functions.

➤ Cardiac valves:

- Mitral valve: Mild annulus calcifications extending to both leaflets with no restriction of both leaflet motion and normal flow pattern.
- Aortic valve: Mild annulus and cusps calcifications with mild AR.
- Tricuspid valve: Mild TR. ESPAP 20 mmHg.
- Pulmonary valve: Normal structure with normal flow, normal main pulmonary artery and its branches.

➤ Pericardium: No apparent pathology.

➤ Intracavitary masses or thrombi: None.

➤ Cardiac septa: Redundant interatrial septum with no flow across it.

### Conclusion:

Sclerocalcific aortic valve with mild AR, calcific mitral valve and LV diastolic dysfunction

Dr. Ahmed Salah, MD



|                                       |                |                  |                                   |               |
|---------------------------------------|----------------|------------------|-----------------------------------|---------------|
| PATIENT NAME<br>نادية حسين ابو العلاء |                |                  | Registered 20-08-2025 11:15:07    | Collected 20- |
| Visit Number<br>29525506952           | Age<br>53 Year | Gender<br>Female | Authenticated 20-08-2025 17:53:39 | Printed 26-   |
|                                       |                |                  | Referred By<br>Prof: جلال         | Cler<br>13    |

| Test Name                      | Result | Unit  | Reference Range                            |
|--------------------------------|--------|-------|--------------------------------------------|
| Tumor Markers                  |        |       |                                            |
| Carcinoembryonic Antigen (CEA) | 7.21   | ng/mL | Non smoker: up to 3.0<br>Smoker: up to 5.0 |

Follow Up Is recommended

Assay Method: CLIA/ECLIA

Results determined by using different methods may not be comparable.

|        |      |      |          |
|--------|------|------|----------|
| CA15-3 | 27.9 | U/mL | 0 - 28.5 |
|--------|------|------|----------|

Assay Method: CLIA/ECLIA

Results determined by using different methods may not be comparable.



A. N. N. N.  
Dr. ABD El-Aziz El-Nokaly  
Professor of Medical Bio-Chemistry  
Faculty of medicine, EL Azhar Univ

Patient Name: **NADIA HUSSEIN**

Date: **Wednesday, July 23, 2025**

### Bilateral Sono- Moammography

- Right LOQ increased density with speculated margin at mammographic ima  
complementary ultrasound there is ill defined mass lesion with acoustic shac  
noted about 28x23mm at 7 o'clock.
- Right axillary multiple suspicious LN noted with thickened cortex and muffle  
hilum, the largest about 3x1.5cm.
- The breast composition is predominantly fibroglandular tissue.
- Normal skin and the subcutaneous tissues lines.
- Normal nipple and retro-areolar region.
- No left axillary lymphadenopathy.
- Complementary US shows:
  - No left focal solid or cystic lesions.

#### Conclusion:

- Right LOQ ill-defined mass lesion as described BIRADs V for biopsy.
- Right axillary multiple suspicious LN.

WITH MY BEST REGARDS  
Dr. Asmaa ABdElmonem

Dr Amr Elsakka Msc

Radiology Department

Dr. IBRAHIM HASSAN

Ph.D. Pathology

دكتور  
إبراهيم حسن  
دكتوراه في علم الأمراض والأورام  
أستاذة التحاليل، الباثولوجية  
بكلية طب الأزهر

Name:

نادية حسين أبو العلا

Date: 12/8/2025

Referred by:

Prof. Dr. Saied Bendary

Ref. No.:

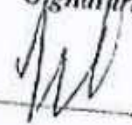
Nature of the specimen: Paraffin block of breast carcinoma for HER-2/neu (C-erbB-2) Immunostaining. Female patient aged 52 years.

Gross description: Sections immunostained for HER-2/neu using Dako autostainer. Dako Ultra View DAB detection system was used.

Microscopic examination: For the determination of HER protein overexpression, only the membrane staining intensity and pattern in the invasive part of tumour are evaluated. HER2 overexpression is interpreted as negative for both scores 0 (no staining is observed or membrane staining in  $< 10\%$  of tumour cells) and 1+ (faint incomplete membrane staining in  $> 10\%$  of tumour cells). The test is interpreted as weakly positive for score 2+ (weak to moderate complete membrane staining in  $> 10\%$  of tumour cells). A strong positive result is interpreted when the score is 3+ (strong complete membrane staining in  $> 10\%$  of tumour cells).

Immunostaining for HER-2/neu: Negative (Score 0).

Signature



25922312: معمل

27865233

معمل : برج النصر .. ميدان باب الشعريّة

Dr. IBRAHIM HASSAN

Ph.D. Pathology

دكتور  
إبراهيم حسن  
دكتوراه في علم الباثولوجيا والأورام  
استاذ التحاليل الباثولوجية  
مكتبة طب الأورام

Name: نادية حسين أبو العلا

Date: 12/8/2025

Referred by: Prof. Dr. Saled Bendary

Ref. No.:

Nature of the specimen: Paraffin block of breast carcinoma for detection of estrogen and progesterone receptor status (ER & PR respectively). Female patient aged 52 years.

Gross description: Sections immunostained for ER & PR using Dako autostainer. Dako Ultra View DAB detection system was used.

Microscopic examination: Sections of the tumour cells show marked nuclear staining for ER. The nuclear reaction for PR, on the other hand, is negative.

Diagnosis: ER Status: Marked positivity (Score 8/8).

PR Status: Negative (Score 0/8).

Signature



معمل 25922312 :  
27865233

معمل : برج النصر - ميدان باب الشعريّة

Dr. IBRAHIM HASSAN

Ph.D. Pathology

دكتور  
إبراهيم حسن  
دكتوراه في علم الباثولوجيا والأورام  
أستاذ المساعد الباثولوجيا  
بكلية طب الأزهر

Name: نادية حسين أبو العلا

Date: 12/8/2025

Referred by: Prof. Dr. Saied Bendary

Ref. No.:

Nature of the specimen: Paraffin block of breast carcinoma for MIB-1 (Ki-67) Immunostaining. Female patient aged 52 years.

Gross description: Sections Immunostained for MIB-1 (Ki-67) using Dako autostainer. Dako Ultra View DAB detection system was used.

Microscopic examination: The infiltrating tumour cells show low Ki-67 (10% of the tumour cells show nuclear positivity for Ki-67).

Immunostaining: Low Ki-67 (10% of the tumour cells show nuclear positivity for Ki-67).

Signature  


معمل : 25922312  
27865233

معمل : برج النصر - ميدان باب الشعريه

Dr. IBRAHIM HASSAN

Ph.D. (PATHOLOGY)

دكتور  
إبراهيم حسن  
دكتوراه في علم الباثولوجيا والأورام  
مركز التحاليل الباثولوجية  
مكتبة طب الأورام

Name: نادية حسين أبو العلا

Date: 29/7/2025

Referred by: Prof. Dr. Mohammed Abol-Wafa

Ref. No.:

Nature of the specimen: Right breast mass, Female aged 52 years.

Gross description: Three cores of tissue measuring 1.5cm long each.

Microscopic examination: Sections reveal infiltration of fibrofatty tissue by irregular groups of malignant epithelial cells exhibiting moderate nuclear anaplasia, mitotic figures and tubular differentiation. The stroma is desmoplastic.

Diagnosis: Right breast mass, tru-cut biopsy: Invasive duct carcinoma, grade II.

N.B. Detection of estrogen and progesterone receptor status, HER-2/neu oncoprotein and MIB-1 (Ki-67) is available on paraffin sections on request.

Signature

معمل : 25922312

27865233

معمل : برج النصر .. ميدان باب الشعريّة

مدير الفرع د. زينب نقيب  
مدير الوحدة ا.د. عبد العزيز النقلي  
طبيب الوحدة ا.د. علا الميمسي  
ا.د. ايات ابراهيم

moamena kamel lab  
معاملة كاملا

د. كامل

د. شريف

|                                       |                  |                                |                                   |                      |
|---------------------------------------|------------------|--------------------------------|-----------------------------------|----------------------|
| PATIENT NAME<br>نادية حسين ابو العلاء |                  |                                | Registered 20-08-2025 11:15:07    | Collected 20-08-2025 |
| Visit Number<br>29525506952           |                  |                                | Authenticated 20-08-2025 17:53:39 | Printed 26-08-2025   |
| Age<br>53 Year                        | Gender<br>Female | Referred By<br>Prof : سيد جلال | Client ID<br>1353                 |                      |

| Test Name         | Result | Unit | Reference Range | Pre |
|-------------------|--------|------|-----------------|-----|
| Hepatitis Markers |        |      |                 |     |

HBs Ag Negative Negative

Assay Method: ECLIA

According to CDC recommendation (2023) triple panel test includes HBs Ag, HBs Ab and HBe Total for initial screening instead test for HBs Ag.

HCV Ab Positive Negative

Screening test to be confirmed by HCV RIBA and HCV viral load by PCR

Assay Method: CLIA

4th generation HIV Ag/Ab Immunoassay. Positive Negative

Assay Method: CLIA/ECLIA



*A. N. N. N.*  
Dr. ABD El-Aziz El-Nokaly  
Professor of Medical Bio-Chemis  
Faculty of medicine, EL Azhar Univ

|                                                     |                                  |
|-----------------------------------------------------|----------------------------------|
| Patient Name : الأستاذ / نادية حسين أبو العلاء محمد | Request No. : 13005840           |
| Patient No. : 27212110101561                        | Request Date : 31/08/2025        |
| Path No. : SR-1197/25                               | Verification Date : 02/09/2025   |
| Sex : Female                                        | Financial Category : نقطة الدولة |
| Age : 52 Y                                          | Department : Out Patient         |
| Referring Doc :                                     | Registered by : Mrs.Mayada Hega  |

### Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Referred slides (Tru-cut biopsy).

#### Gross Examination:

Five referred slides labelled by patient name were received, including 1 H&E stained slide and 4 immunostained slides for (ER, PR, HER2neu & Ki67).

#### Microscopic Examination:

- \* Histological type: Invasive duct carcinoma.
- \* Grade 2 (differentiation 2, nuclear pleomorphism 2, mitosis 2).
- \* DCIS: Absent.
- \* Angioinvasion: Absent.
- \* Perineural invasion: Absent.
- \* IPT:
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Moderate nuclear reaction in 1% of tumor cells (score 4/8).
- Her2: Negative membrane staining in tumor cells (score 0).
- Ki67%: 10%.

#### Diagnosis:

Right breast mass, Referred slides (Tru-cut biopsy):

- Invasive duct carcinoma, Grade 2.

#### IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 4/8).
- Her2: Negative (score 0).
- Ki67%: 10%.

Verified by :

Dr.Duaa Mohamed

Printing Date : 06/09/2025 15:05:28

Page 1 of 2

Printing By : .marwa moham

Patient Name : الأبرقازو / فادية حسين أبو العلا محمد

Patient No. : 27212110101561

Path No. : SR-1197/25

Sex : Female

Age : 52 Y

Referring Doc :

Request No. : 13005840

Request Date : 31/08/2025

Verification Date : 02/09/2025

Financial Category : نفقة الدولة - فاروسي

Department : Out Patient

Registered by : Mrs. Mayada Hega

### Pathology Report

\* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

\* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr. Duaa Mohamed

Printing Date : 06/09/2025 15:05:28

Page 2 of 2

Printing By : marwa moham



غرفة رقم (4) .. الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر

|      |                                        |          |           |
|------|----------------------------------------|----------|-----------|
| Name | Mrs. Nadia Hussein Abou Al-Ela Mohamed |          |           |
| Ref. | مستشفى الحسين الجامعي                  | Date     | 21/8/2025 |
| ID   | 9925017160                             | Acc. No. | 240258343 |

**Abdomen & Pelvis:**

- Enlarged metabolic hyperactive left para-aortic lymph node measuring 1.5 X 1 cm SUVmax.4.9. Another metabolic active lymph node right iliac fossa SUVmax.13.9.
- Average size of the liver with non-uniform activity yet no FDG hypermetabolic focal lesions mostly blood pooling. For dynamic MRI study of the liver assessment if warranted.
- No biliary tree dilatation.
- Average size of the spleen with no focal active lesions.
- Normal appearance of pancreas (no sizable active masses), adrenals and both kidney.
- Congested mesentery with strandy reticulations.
- Normal physiological biodistribution over the bowel (non-uniform bowel uptake with sizable masses) & urinary tracts.

**Surveyed skeleton:**

- No tracer active lytic or sclerotic osseous lesions seen.
- Faint marrow uptake of SUVmax. 2.5 yet no tracer active lytic or sclerotic osseous lesions seen, likely hematopoietic, for CBC correlation and follow up.
- Spondylodegenerative changes of the spine.

**Opinion:**

- Right breast pathological proven metabolic active mass lesion with right axillary metastatic deposits as well as left para-aortic lymph node and right iliac nodal deposits.
- Rest of the surveyed body is free from any other FDG hypermetabolic neoplastic masses.

Thanks for your referral;

Prof. Dr. Hani Ahmed Sami

Prof. Dr. Essam Ali El-Sheikh

Typed: H.S. 25/08/25 3:27:41 PM Dr. M.H.

Dr. Ahmed Kandeel

MD Nuclear Medicine

MDT FORM

10 / 9 / 20 25 G

|                   |                  |                |         |
|-------------------|------------------|----------------|---------|
| Name              | نادية م. أبو بكر | MRN            | 24577   |
| Age               | 52 y.            | ID             | 272121  |
| Menopausal status | Post menopause   | Phone          | 0101391 |
| NRS SCORE         |                  | Marital status | Married |
| BMI               |                  | Family history | Neg.    |
| CO morbidity      | Med. Free.       | REASON OF MDT  | New     |
|                   |                  | HCV +          |         |
|                   |                  | HIV +          |         |

Clinical Data

6751211.1.1071

BX:

IDC GII

ER:

8/8

PR:

4/8

HER 2:

0

Ki67:

10%

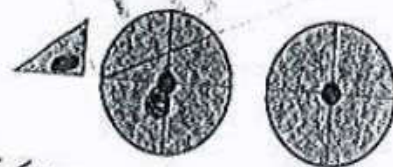
Sonomammography:

Rt LOQ ill defined mass lesion.  
2.8X2.3 cm at 7 o'clock - L.N 3X1.5 cm.

Mets Workup:

PET-CT → Free **except**  
Lt para aortic L.N (1.5X1 cm SUV 4.9)  
Rt iliac fossa L.N (SUV 13.9) 9.2

R



TNM  
2 2

Recommendations:

2.2

MDT OUTCOME & PLAN

- review of PET-CT film with our Radiology to III L.V.
- confirm on Antihormonal treat PCR
- Start AC X 2 Tazol X 12 weeks
- CESM to decide type surgery I C

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

Nutrition

PSI

|      |                                        |          |           |
|------|----------------------------------------|----------|-----------|
| Name | Mrs. Nadia Hussein Abou Al-Ela Mohamed |          |           |
| Ref. | مستشفى الحسين الجامعي                  |          |           |
| ID   | 9925017160                             | Date     | 21/8/2025 |
|      |                                        | Acc. No. | 240258343 |

### $^{18}\text{F}$ -FDG PET/CT Scan

**Clinical history Indication of the study:** Status evaluation study for a case with pre history of invasive duct carcinoma 7/2025 pathological proven.

#### Technique:

- Total body PET scan was done 40 minutes after IV administration of 300 MBq of  $^{18}\text{F}$ -F
- CT scanning was performed simultaneously without IV contrast material injection.
- Semi-quantitative analysis would be done for selected ROI.s & the normal threshold to be the normal blood pool liver SUV max. < 2

#### Previous exams:

- Review of the previous.

#### Head & Neck:

- Normal physiological biodistribution over scanned portion of the brain.  
(N.B; not all brain metastatic lesions could be detected on  $^{18}\text{F}$ -FDG PET/CT due to physiologic uptake in brain. MRI brain is suggested in cases of clinical suspicion).
- Normal naso-/oro-pharynx, larynx, thyroid and salivary glands.
- No metabolically active cervical lymphadenopathy.

#### Torso:

##### Thorax:

- Multiple metabolic active variable sized right axillary lymph nodes with SUVmax.16. measuring 3 X 2.5 cm, large lobulated metabolic active right breast mass measuring 2.7 cm, eliciting SUVmax.8.0. No left axillary lymph node enlargement or activity. No breast masses.
- Bilateral tracer starved atelectasis and sub-pleural reticulations.
- No FDG hypermetabolic pulmonary nodules or masses seen.
- No enlarged FDG hypermetabolic hilar or mediastinal L.Ns seen.
- No pleural or pericardial effusion seen.
- Cardiac FDG uptake appears physiologic. No FDG-avid vascular abnormalities or me evidence of inflammation.

|                 |                       |                 |                |
|-----------------|-----------------------|-----------------|----------------|
| Patient Name    | نادية حسين ابو العلا  | Case No.        | 338921         |
| Sex/Age         | Female 53 Y           | Patient No.     | 24212          |
| Registered Date | 28/6/2026 09:43:57 AM | Referred By Dr. | Surgery        |
| Validation Date | 09/8/2026 02:11:43 PM | Branch          | Gustave Roussy |
| Reporting Date  | 27/8/2026 03:28:25 PM | Pathology No.   | 2038/26        |

## Pathology Report

**Clinical Data:** Right breast cancer, post-NACT.

**Nature of the specimen:** Right modified radical mastectomy.

### **Gross Examination:**

- \* Right modified radical mastectomy specimen measured 30x17x10 cm, with covering skin ellipse measured 27x15 cm. Nipple & areola looked unremarkable grossly.
- \* Serialling showed a tumor bed site 9.5x6x5.5 cm, located 8 cm from the least deep margin.
- \* Axillary fat entangling 13 lymph nodes, the largest measured 2.0x1.0x0.5 cm.
- \* **Block designation:**
- \* 12 Blocks:
- 1:6- T.B.S., 7- Deep margin., 8- Skin., 9:12- ALNs.
- Gross was done by; Dr. Aya Ezzat.

### **Microscopic examination:**

- **Histological type:** Residual viable invasive duct carcinoma.
- Tumor size: 4.5x4.5 cm.
- Tumor Cellularity: 70%.
- Grade: 2 (differentiation 3, nuclear pleomorphism 2, mitosis 1).
- Ductal carcinoma in situ: Absent.
- Nipple and areola: Free.
- Resection Margins: Free deep margin.
- Lymph node involvement: Positive viable metastatic deposits in thirteen out of thirteen lymph nodes (13/13) (The largest metastatic focus measuring 1.2 cm, with extra-nodal extension and with therapy effect).
- Histologic response to therapy: Pathological partial response.

### **Diagnosis:**

**Right Breast Cancer (Post-NACT); Right Modified Radical Mastectomy:**



جوستاف رويسى القى للامراض - مصر مستشفى الجولوى - (987) - رقم الجوال - مصر  
 Room No. (4) - First Floor-Building No. 987- Nile Corniche- Minr El Qadima - Cairo, Egypt

|                 |                       |                 |                |
|-----------------|-----------------------|-----------------|----------------|
| Patient Name    | نادية حسين ابو الللا  | Case No.        | 338921         |
| Sex/Age         | Female 53 Y           | Patient No.     | 24212          |
| Registered Date | 28/6/2026 09:43:57 AM | Referred By Dr. | Surgery        |
| Validation Date | 09/8/2026 02:11:43 PM | Branch          | Gustave Roussy |
| Reporting Date  | 27/8/2026 03:28:25 PM | Pathology No.   | 2038/26        |

- Residual viable invasive duct carcinoma, Grade 2 (Cellularity 70%) (Actual tumor size 4.5x4.5 cm).
  - Ductal carcinoma in situ: Absent.
  - Nipple and areola: Free.
  - Deep margin: Free.
  - Axillary lymph nodes: Positive viable metastatic deposits in thirteen out of thirteen lymph nodes (13/13) (The largest metastatic focus measuring 1.2 cm, with extra-nodal extension and with therapy effect).
  - Histologic response to therapy: Pathological partial response.
- Stage: ypT2, ypN3.
- RCB: Tumor cellularity 70%, RCB= 4.44, RCB class III.

Signature

Prof. Dr. Maha Guimei.


 طابق رقم (4) - الطابق الأول - مبنى رقم (1987) - مبنى القيد - مصر الجديدة - القاهرة 5 - مصر  
 Room No. (4) - First Floor-Building No. 987- Nile Courtyard - Near El Qudima - Cairo, Egypt

جمهورية مصر العربية  
بطاقة تحقيق الشخصية



نادية

حسين ابو العلا محمد

مساكن العبد القديم ب ٣٩ م ١

اول السلام - القاهرة

١٩٧٢/١٢/١١

٢٧٢ ١٢ ١١ . ١ . ١٥ ٦١

IT5322450



٢٠٢١/٠٢ ٢٧٢١٢١١.١.١٥٦١

ربة منزل

أرملة

مسلمة

أنثى



البطاقة سارية حتى ٢٠٢٨/٠٢/٠٢



مستشفى  
الزهراء  
بغداد

9-7



مبادرة رئيس الجمهورية  
ل دعم صحة المرأة المصرية

وزارة الصحة  
بغداد

09/06/2026

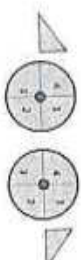
## MDT FORM

|                    |                                  |                         |                |
|--------------------|----------------------------------|-------------------------|----------------|
| Name:              | Nadia Hussien Abo El Ela Mohamed | MRN:                    | 7962           |
| Age:               | 54 Y                             | ID:                     | 27212110101561 |
| Menopausal Sta:    | POST                             | Phone:*                 | 01013912023    |
| NRS SCORE:         |                                  | Marital Status:         |                |
| BMI:               |                                  | Family History:         | -VE FH         |
| Gender             |                                  | Reason Of MDT:          |                |
| CO Morbidity:      | M FREE                           | POST OP                 |                |
| Surgical History:* | APPENDECTOMY                     | ECOG Performance Status | 0 1 2 3 4      |

## Clinical Data:

R

L



T N M

## BX:

ER: 8/8

PR: 4/8-

HER2: 0

KI67:

## History:

patient finished NACT 3X AC 12 WKS Taxol ended in 3/4/2026

## Sociodemography:

SP

RT MRM POST NACT

RESIDUAL VIABLE INVASIVE DUCT CARCINOMA

RESECTION MARGIN &gt;&gt; FREE DEEP MARGIN

LN ( 13/13)

PATHOLOGICAL PARTIAL RESPONSE

Biomarker/Molecular

## Metis Workup:

## Clinical Staging:

## Pathological Staging:

## Recommendations:

ARCHIVE PATHOLOGY REPORT

## MDT Outcome &amp; Plan

PORT

AI FEMARA/7YS

ADJ CDK4/6 BY NGO

CT CAP /AMS

ZOMETA /6MS

## SIGNATURE

|                  |         |           |           |
|------------------|---------|-----------|-----------|
| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY |
|                  |         |           |           |

Signature of Medical Oncologist